

Medicare won't pay for your mother's nursing home. Her house will.

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Most Americans believe Medicare covers nursing home care. It doesn't. It pays for up to 100 days of skilled nursing after a hospital stay, and nothing for the daily help with bathing, eating and dressing that most residents actually need. Families usually learn this in a hospital discharge office, holding a bill.

More than half of Americans turning 65 this year will need long-term care. A private nursing home room now runs a median of \$129,575 a year. The median household over 65 earns about \$57,000. So here is what happens to a middle-class family, step by step.

First, you pay until you're broke. Among residents who enter paying privately, one in six spends down to Medicaid during their stay, in an average of just six months. Second, you qualify as poor. In most states, Medicaid won't pay until you hold no more than \$2,000 in countable assets. Third, the state takes the house. Federal law requires states to recover Medicaid's costs from your estate after you die, and for most working families the estate is the house.

The rich don't climb that ladder. With an elder-law attorney and enough lead time, you move your assets into an irrevocable trust, wait out Medicaid's five-year lookback, and qualify with your wealth intact. It's legal and routine. Firms advertise the fee as "less than one month of nursing home care." One elder-care lawyer says it outright on his own blog: paying for long-term care "is really a problem for the middle and working classes, not the rich."

Who's on the other end? Medicaid is the primary payer for 63% of nursing home residents, and 73% of nursing homes are for-profit. A lot of that public money doesn't stay in the building. Nearly three-quarters of homes pay "rent" and "management fees" to companies their own owners control. A 2024 National Bureau of Economic Research paper found homes hide at least 63% of their profits this way.

Wall Street noticed. Real estate investment trusts now hold stakes in one in six nursing homes and paid more than \$7 billion in dividends in 2024. One of the largest, CareTrust, booked a 67% profit margin last year. A KFF Health News investigation this spring found a home paying its REIT landlord more than \$1 million a year in rent while the home itself ran a deficit, and found residents in these portfolios receiving up to an hour less nursing care

per day than the national average. When private equity buys a nursing home, a landmark study of 7 million Medicare patients found, short-term mortality rises 10% and billing rises 19%, most of it to taxpayers.

The nursing assistant turning your mother every two hours makes a median of \$20 an hour and turnover in her job runs close to 100% a year. She is not the problem. The people above her are. A landlord collecting rent from a home running a deficit, with Medicaid covering the gap and residents getting less care, isn't running a business. He's running a toll booth on the last years of someone's life, and the toll is paid twice: first by the family, then by the taxpayer.

The federal government tried to do something. In 2024 it set a minimum staffing standard. The industry's lobby spent nearly \$17 million fighting it. In December, the rule was repealed.

None of this is inevitable. Germany and Japan run mandatory public long-term care insurance that everyone pays into and everyone can use, no spend-down required. The United States passed its own version in 2010, the CLASS Act. Congress repealed it in January 2013 before it enrolled a single person. "Go broke first" is a policy choice.

And it's a choice being made against young people. Households under 40 already hold just 6.6% of the nation's wealth, down from 12% in 1989. The house that would have been a first down payment is instead becoming a landlord's rent check. Unpaid family caregiving, mostly by adult children, was worth an estimated \$600 billion in a single year. And the generation paying the Medicaid bills today will face the same \$2,000 rule tomorrow, with less saved.

Congress can act this session. Restore the staffing floor. Require any facility taking Medicaid to disclose every related-party payment in plain English. And put a public long-term care insurance program back on the table, as a bipartisan bill to bar Medicare payments to private-equity-owned homes has already begun to do.

I run a nonprofit that has taught more than 100,000 young people about money. I can teach them to save. I cannot teach them to out-save a system built to take the house.

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